

Non-autoimmune diseases

Main differential diagnoses for DH: non-autoimmune diseases

Scabies

Scabies may closely mimic the pruritic, vesicular, and papular lesions of dermatitis herpetiformis (DH), particularly in the early stages. The presence of burrows, preferential involvement of finger webs, wrists, elbows, and genitalia, and intense nocturnal itching are suggestive of scabies. A definitive diagnosis is achieved by identifying *Sarcoptes scabiei* mites, eggs, or fecal matter in skin scrapings examined under microscopy.

Irritant or allergic contact dermatitis

These conditions can present with pruritic papules, vesicles, and erythematous plaques, resembling DH, especially when distributed on the elbows, knees, or buttocks. However, contact dermatitis typically correlates with exposure to specific allergens or irritants and lacks the symmetrical, chronic-relapsing pattern seen in DH. Patch testing can aid in identifying the responsible agent.

Atopic dermatitis

Atopic dermatitis may present with pruritic, eczematous lesions that could be confused with the eczema-like variant of DH. However, atopic dermatitis usually affects flexural areas, has a personal or family history of atopy, and lacks the characteristic immunopathological findings of DH, such as granular IgA deposits at the dermal papillae.

Arthropod bite reactions

Reactions to insect bites can manifest as grouped or linearly arranged papules and vesicles (often referred to as "breakfast, lunch, and dinner" lesions), which may resemble the agminated vesicles of DH. A detailed history of outdoor exposure and the transient nature of bite reactions help differentiate them from the persistent, symmetrical lesions of DH.

Pityriasis lichenoides chronica

This condition presents with recurrent crops of small, scaly, reddish-brown

papules that may occasionally have a vesicular appearance. It typically affects the trunk and limbs and may be mistaken for the papulovesicular stage of DH. However, it lacks the intense pruritus and characteristic IgA deposition seen in DH.

Dyshidrotic eczema

Also known as pompholyx, this condition is characterized by deep-seated vesicles on the palms, soles, and lateral aspects of the fingers—sites also commonly affected in DH. However, dyshidrotic eczema is not associated with gluten sensitivity and shows spongiosis and vesicle formation on histology without neutrophilic microabscesses or IgA deposits.

Drug eruptions

Certain drug-induced eruptions (e.g., those caused by vancomycin, amphetamines, or anticonvulsants) can present with pruritic papulovesicular lesions resembling DH. A thorough medication history and resolution of lesions upon drug withdrawal support the diagnosis of a drug eruption. Additionally, these lack the specific immunopathological features of DH.